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Multimodal integration reveals complementary imaging and genetic predictors of coronary artery disease risk

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github.com/Devanshpandey/Multimodal-integration-for-coronary-artery-disease-risk-prediction

Cardiac MRI embeddings and polygenic risk capture independent CAD risk — combining them identifies a subgroup with ≈23× the cohort event rate.

Colour key ■ imaging (cardiac MRI) ■ extra-cardiac imaging ■ genetics (PRS) ■ clinical / guideline scores ■ combined model

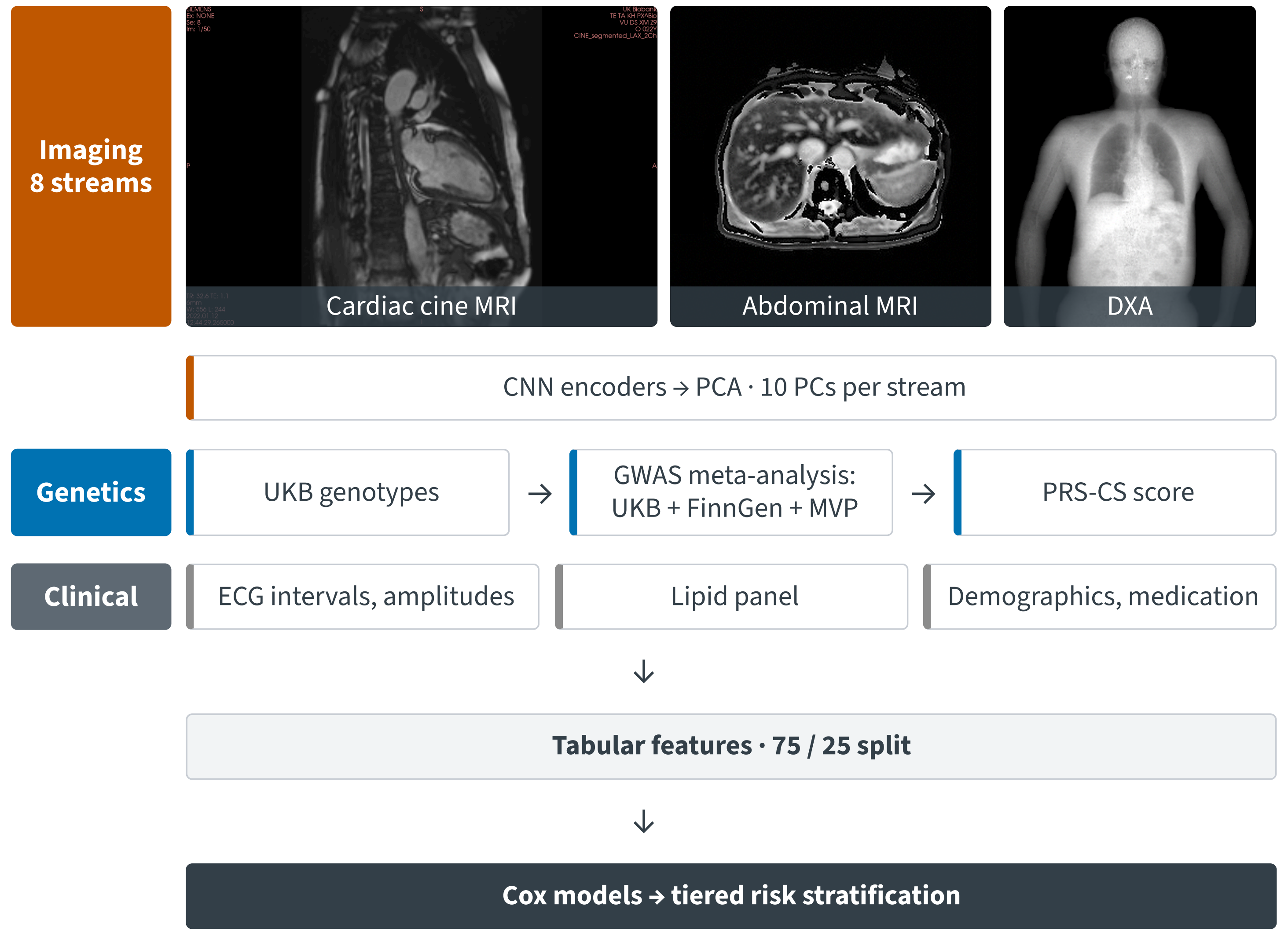
WHY & HOW

UK Biobank imaging cohort, three data domains, one model

Background & questions

- Leading cause of death; ~50% of CAD first presents as a heart attack.
- Clinical scores omit genetics and imaging; how much imaging and genetic risk overlap was unknown.

1. Which imaging modalities add to clinical, metabolic, ECG and genetic factors?
2. Overlap between imaging-derived and polygenic risk
3. Sequential integration: discrimination, calibration, stratification


Imaging 8 streams

Cardiac cine MRI

Abdominal MRI

DXA

CNN encoders → PCA · 10 PCs per stream

Genetics

UKB genotypes → GWAS meta-analysis: UKB + FinnGen + MVP → PRS-CS score

Clinical

ECG intervals, amplitudes

Lipid panel

Demographics, medication

↓

Tabular features · 75 / 25 split

↓

Cox models → tiered risk stratification

Cardiac MRI does not image coronary atherosclerosis; its signal reflects remodeling, vascular stiffness and cardiometabolic burden.

Outcome = clinically recognized CAD (MI, angina, revascularization); prevalent cases excluded.

42,351
UK Biobank imaging participants

677
incident CAD (1.6%)

4.6 yr
median follow-up

211,755
cardiac cine MRI series

127,053
2D images (liver, pancreas, DXA)

10,597
held-out test set (178 cases)

Methods in brief

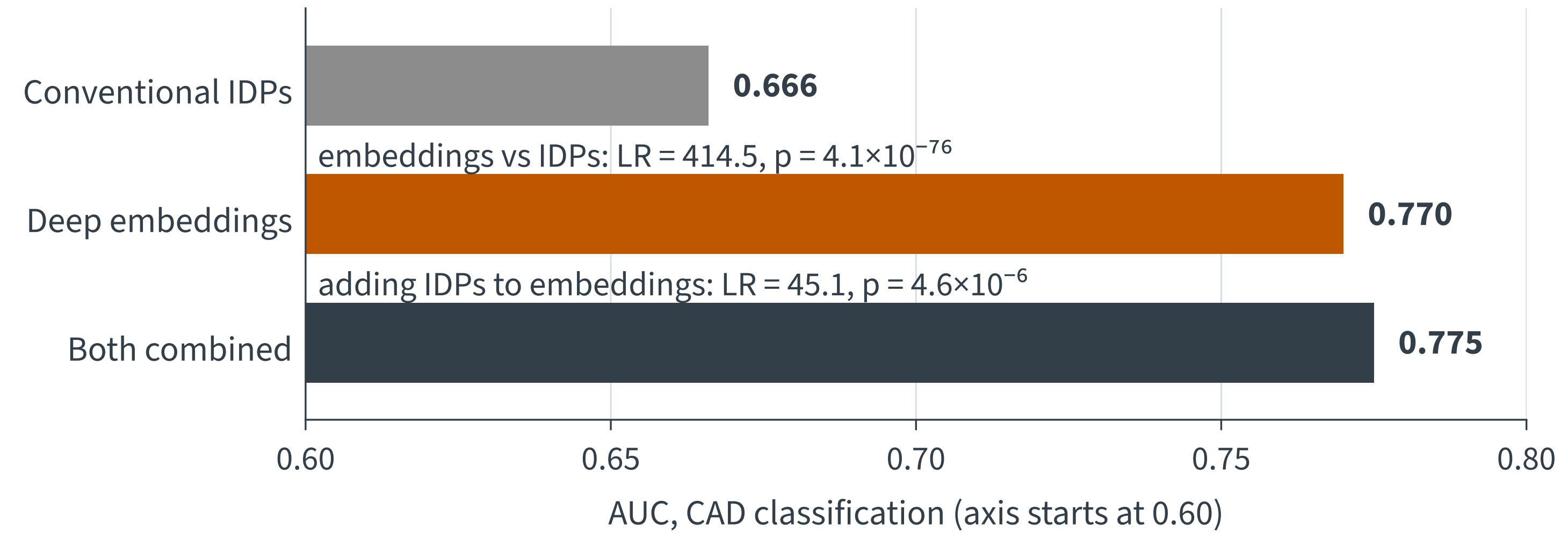
- Encoders fine-tuned on balanced case–control training data only.
- Embeddings → PCA, 10 PCs per stream (80 total).
- PRS-CS, European meta-GWAS: OR 1.87 per SD, AUC 0.766 (with age, sex, 20 PCs).
- XGBoost + nested Cox; imaging risk score = cross-fitted ridge Cox.

IMAGING REPRESENTATIONS

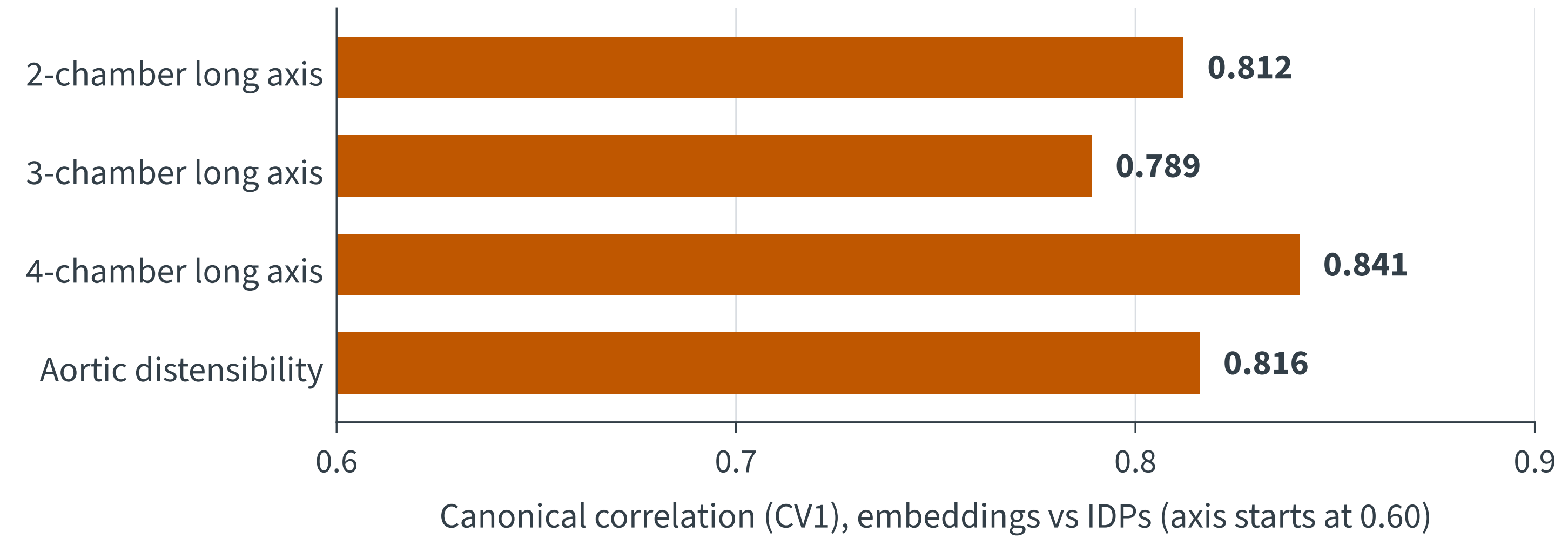
Deep embeddings beat handcrafted cardiac phenotypes

0.770 vs 0.666

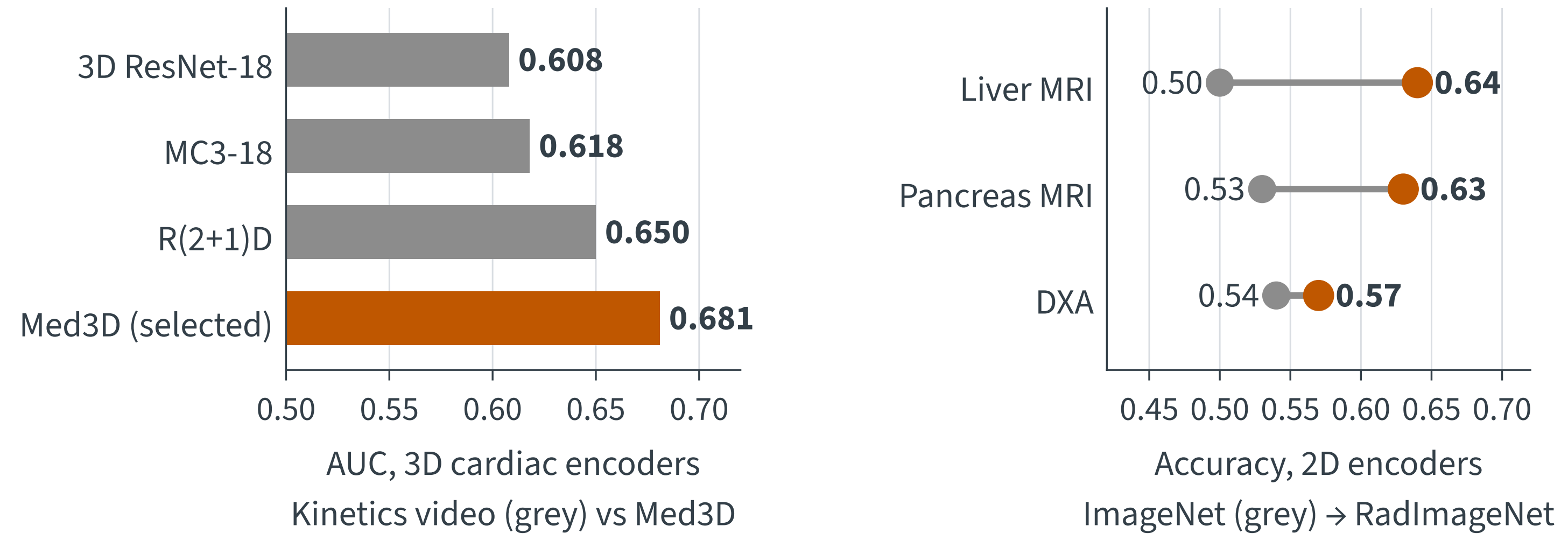
AUC, deep embeddings vs conventional imaging-derived phenotypes (IDPs); both combined 0.775



Embeddings outperform IDPs; adding IDPs barely helps (logistic models; performance plateaus at ~5 PCs per modality; Fig. 2a,b).



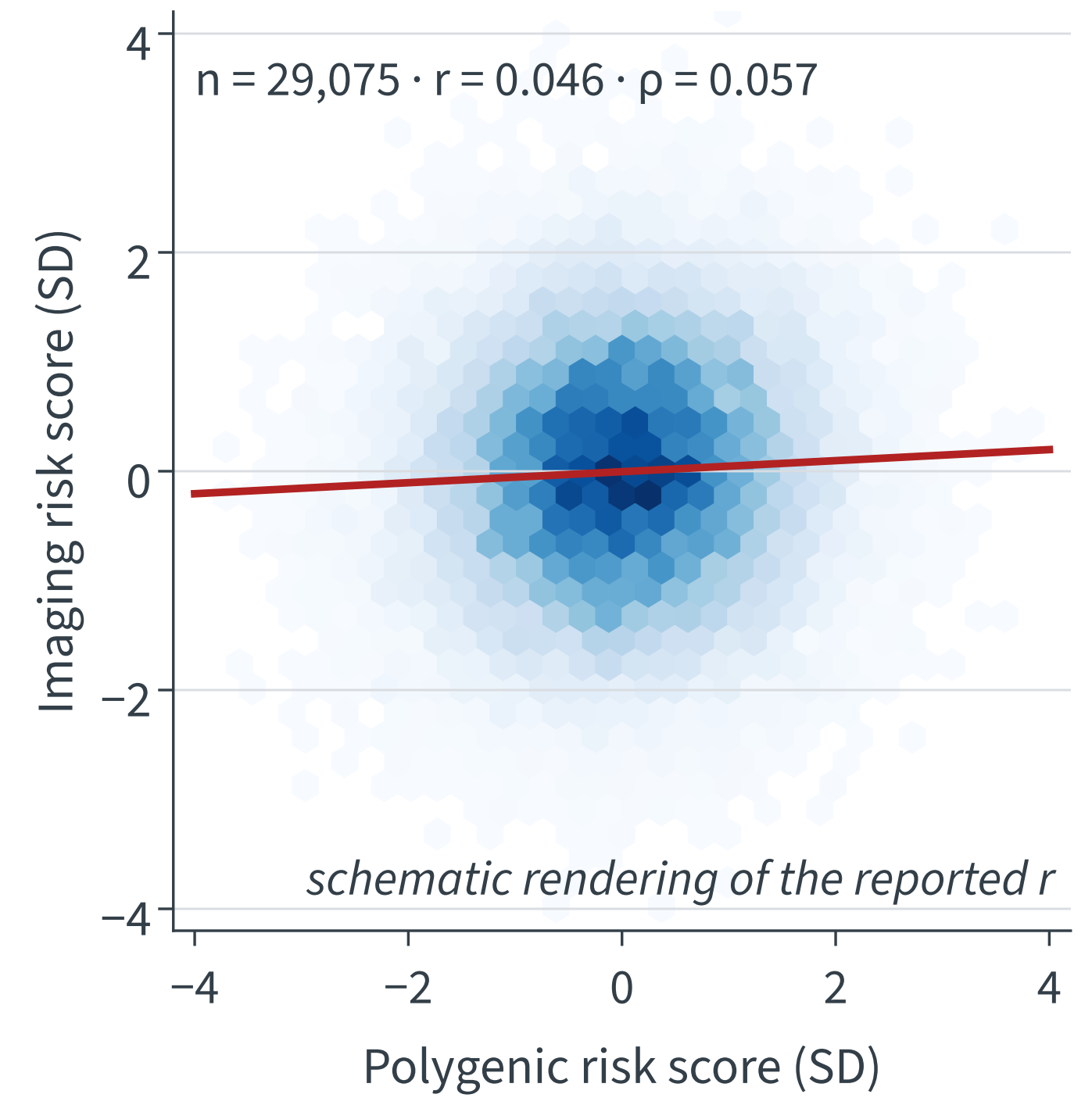
Embeddings encode physiology: CV1 loads on stroke volume, ejection fraction, myocardial mass and wall thickness (Fig. 2c).



Medical pretraining wins: Med3D for the cardiac 3D encoders, RadImageNet for liver, pancreas and DXA (axes from 0.50 / 0.45; Supp. Fig. 3b, 4b).

IMAGING VS GENETICS

Imaging and genetic risk are nearly independent



r ≈ 0.05

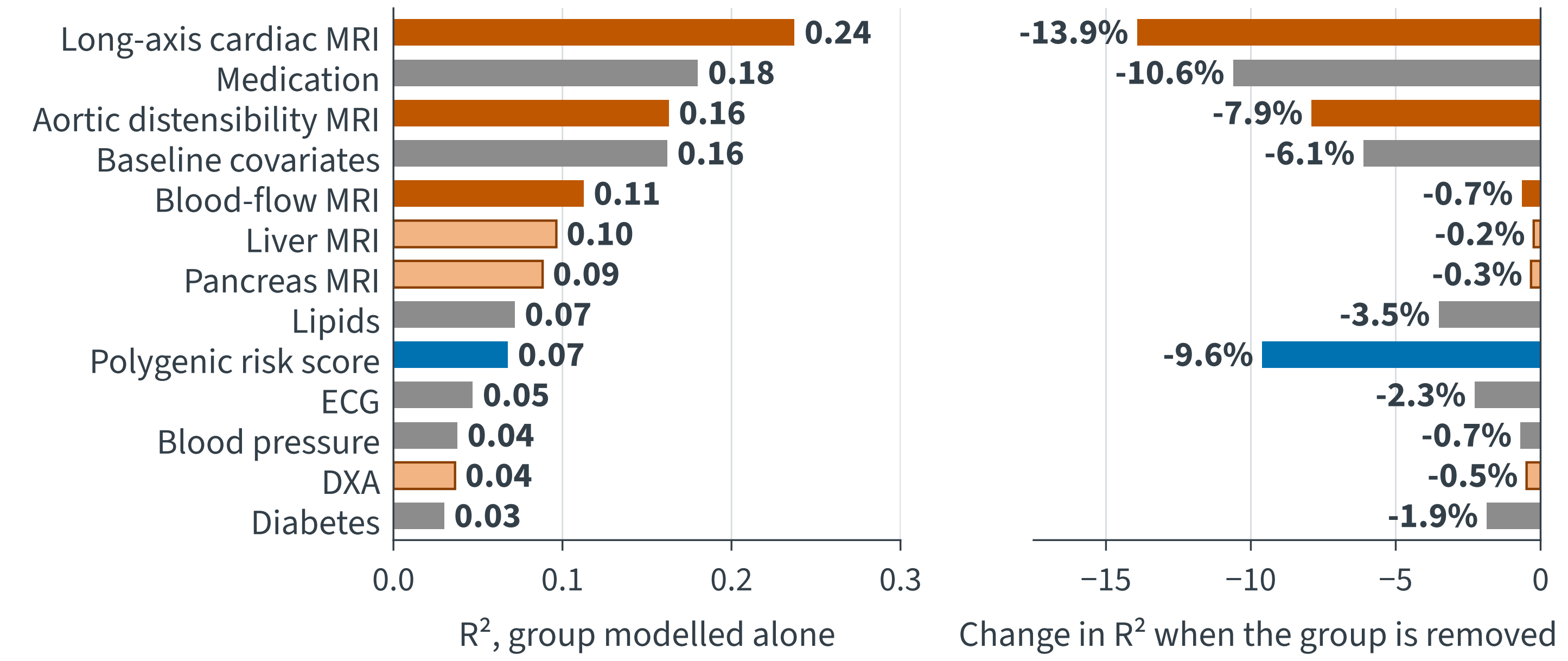
Pearson r, imaging vs polygenic risk score (ρ = 0.057); $R^2 < 1\%$ shared — the two overlap very little (n = 29,075; Fig. 3c–e).

Partial R^2 of imaging risk explained by

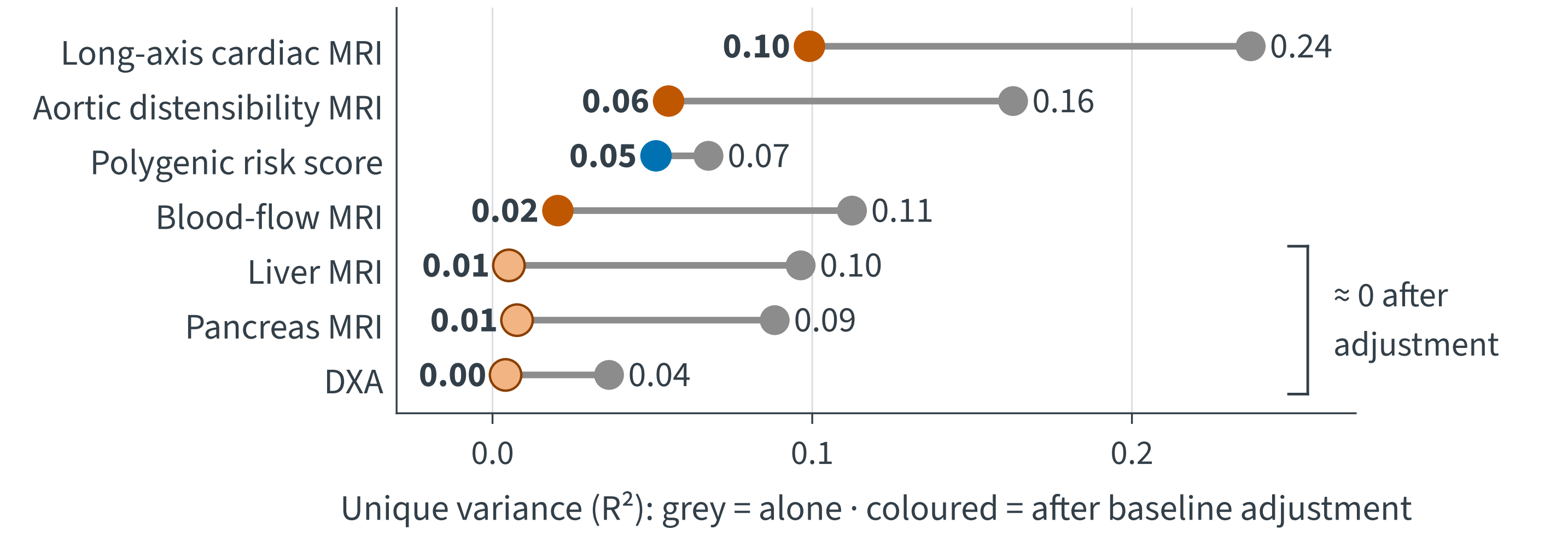
0.15% genetics (PRS)

38.4% demographics

14.4% clinical factors



Cardiac imaging, medication and PRS are each non-redundant. Left: R^2 of each group alone; right: R^2 drop when the group is removed from the full model (Fig. 3a,b).



Only long-axis, aortic distensibility MRI and PRS retain unique variance after clinical adjustment (Fig. 3f).

INTEGRATION & STRATIFICATION

Sequential integration finds very-high-risk people

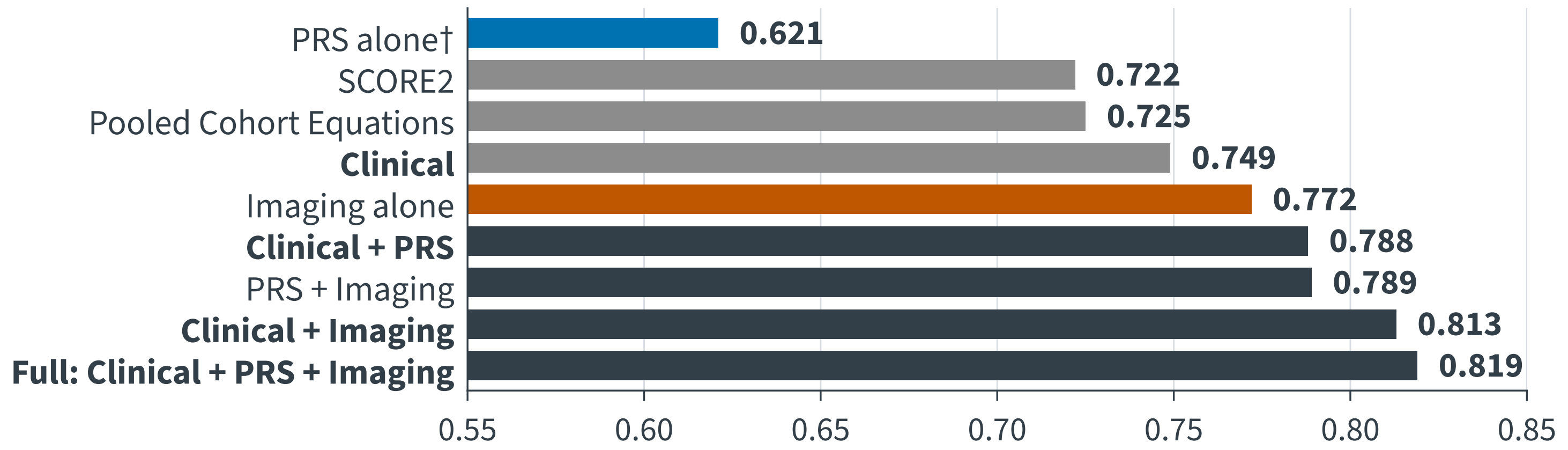
36.8%
5-yr CAD incidence, top quintile of both imaging and genetic risk within Tier 3; cohort 1.6%

0.634
continuous NRI vs clinical (95% CI 0.534–0.734)

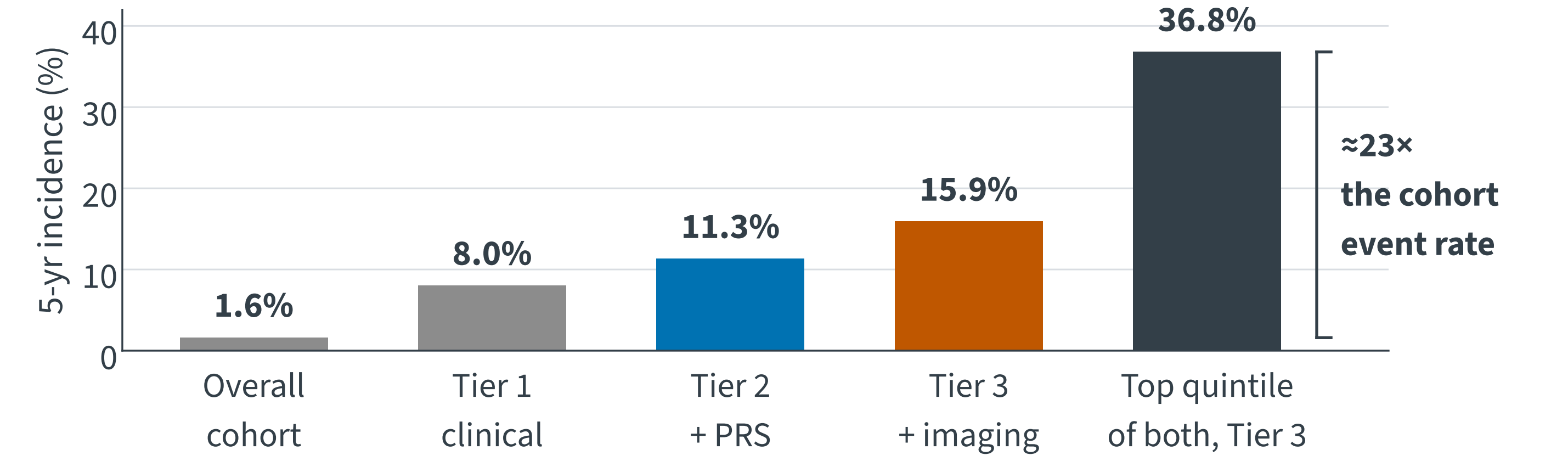
9.5%
prediction accuracy (IPA), full model; clinical 5.5%

86.8 yr
median heart age, Tier 3; cohort 55.3 yr

0.99–1.08
calibration slopes, nested models



Each layer adds discrimination; imaging is among the strongest independent predictors (HR ≈ 2.1 per SD). Held-out C-index; axis from 0.55; bold = nested. HR per SD, full model: imaging ≈ 2.1, PRS ≈ 1.7, demographics ≈ 1.4, lipids ≈ 1.1. †PRS alone = score without age/sex; AUC 0.766 (column 1) includes age, sex, 20 PCs.



Sequential tiers enrich risk far beyond clinical stratification (5-yr incidence, held-out set). Leave-one-centre-out: AUC 0.785–0.822, C-index 0.73–0.76; discrimination is stable across the four UK Biobank imaging centres (site-level robustness, not external validation).

Conclusions

- Imaging and polygenic risk are nearly uncorrelated: complementary axes.
- Long-axis cine and aortic distensibility MRI carry the incremental signal; blood-flow, liver, pancreas and DXA add little once clinical variables are included.
- Clinical → PRS → imaging integration isolates a 36.8% 5-yr-risk subgroup.
- A framework for complementary risk — not yet a clinical screening tool.

Limitations

- No CAC/CTA in UK Biobank: no calcium-score benchmark.
- European-ancestry, complete-case, healthier-than-general cohort; covariates measured at imaging, so part of imaging's gain may reflect temporal proximity.
- Not externally validated; observational — prospective trials needed.